

Cetuximab (Erbix[®])

Cetuximab (Erbix[®]) is a cancer drug treatment. It is used to treat bowel (colon and rectal) cancer and head and neck cancer.

On this page

- [What is cetuximab \(Erbix[®]\)?](#)
- [How cetuximab is given](#)
- [About side effects](#)
- [Side effects while treatment is being given](#)
- [Very common side effects](#)
- [Other side effects](#)
- [Other important information](#)
- [How we can help](#)

What is cetuximab (Erbix[®])?

Cetuximab is also called Erbix[®]. It is a type of targeted therapy drug called a monoclonal antibody.

Cetuximab is used to treat bowel ([colon](#) and [rectal](#)) cancer and [head and neck cancer](#). It may sometimes be used to treat other cancers.

Cetuximab may be used to treat:

- **Advanced bowel cancer** (colon or rectal cancer) after tests to find out what gene changes (mutations) the cancer cells may have.
 - It can be given on its own, or with chemotherapy if tests show the cancer cells have a normal RAS gene.
 - It may sometimes be given with a targeted therapy. This is when tests show the cancer cells have a normal RAS gene and a gene change called a BRAF V600E mutation.
- Head and neck cancer that has spread.
 - It may be given with chemotherapy or radiotherapy.

It is best to read this information with our general information about [targeted therapy drugs](#) and the [type of cancer](#) you have.

Your cancer will talk to you about this treatment and its possible side effects before you agree ([consent](#)) to have treatment.

More information about this treatment

This information is correct at time of publishing. But sometimes the types of cancer this treatment is used for, or treatment side effects, may change between revision dates.

You can talk to your cancer team if you want more detailed information about this treatment. Or visit the [electronic Medicines Compendium \(eMC\)](#) website, which has patient information leaflets (PIL) for individual drugs.

How cetuximab is given

You usually have cetuximab at a day unit or clinic as an outpatient. You may have it on its own or with radiotherapy, chemotherapy or another targeted therapy, depending on the type of cancer you have.

During a course of treatment, you will meet someone from your cancer team, such as a:

- cancer doctor
- specialist nurse
- specialist pharmacist.

This is who we mean when we mention doctor, nurse or pharmacist in this information.

During your course of treatment, you will have regular blood tests. This is to check that it is safe for you to have treatment.

You will speak a doctor, nurse or pharmacist before you have treatment. They will talk to you about your blood results and ask how you have been feeling. If your blood results are okay, the pharmacy team will prepare your targeted therapy.

You have cetuximab as a drip into a vein (intravenous infusion). The nurse will run the infusion through a pump. This will give you the treatment over a set time. Your nurse will give your treatment through 1 of the following:

- a [cannula](#) – a short, thin tube the nurse puts into a vein in the arm or hand
- a [central line](#) – a fine tube that goes under the skin of the chest and into a vein close by
- a [PICC line](#) – a fine tube that is put into a vein in the arm and goes up into a vein in the chest
- an [implantable port](#) (portacath) – a disc that is put under the skin on the chest or arm and goes into a vein in the chest.

You have the first infusion over 2 hours and then stay in the clinic for about 1 hour after. This is to check you do not have a reaction to the drug. If you do not have any problems, your next treatment can be given at faster rate depending on dose.

Your course of treatment

You may have cetuximab once a week or every 2 weeks. How long you have it for depends on the type of cancer you have. If you are having it with other cancer drugs on the same day, you usually have cetuximab first.

If you are having it with other cancer drugs on the same day, you usually have cetuximab first.

Your cancer team will discuss your treatment plan with you. They may also give you anti-sickness drugs and other medicines to take home. Take all your capsules or tablets exactly as they tell you to.

About side effects

We explain the most common side effects of this treatment here. We also include some that are less common.

You may get some of the side effects we mention, but you are unlikely to get all of them. And you may have some side effects, including rarer ones, that we have not listed here.

Other cancer treatments may cause different side effects. If you are also having other cancer treatment, you may have other side effects.

Always tell your doctor, nurse or pharmacist about any side effects you have. They can give you:

- drugs to help control some side effects
- advice about managing side effects.

It is important to take any drugs exactly as explained. This means they will be more likely to work for you.

Serious and life-threatening side effects

Some cancer treatments can cause serious side effects. Sometimes, these may be life-threatening. Your doctor, nurse or pharmacist can explain the risk of these side effects to you.

Contact the hospital

Your doctor, nurse or pharmacist will give you 24-hour contact numbers for the hospital. If you feel unwell or need advice, you can call at any time of the day or night. Save these numbers in your phone or keep them somewhere safe.

Side effects while treatment is being given

Some people may have side effects while they are being given this treatment or shortly after they have it.

Allergic reaction

Some people have an allergic reaction to cetuximab. This can happen while having the infusion or for up to 1 hour afterwards. You will be given drugs before the infusion to reduce the chance of a reaction happening.

A reaction is usually mild, but rarely it can be more severe. If you have a reaction, the nurses can usually treat it by slowing or stopping the infusion. You will then have the rest of your treatments more slowly.

Your nurse will check you for signs of a reaction during your treatment. Always tell them straight away if you feel unwell or have any of the following symptoms:

- feeling hot or flushed
- a skin rash
- itching
- shivering
- feeling dizzy
- a headache
- feeling breathless or wheezy
- swelling of your face or mouth
- pain in your back, tummy (abdomen) or chest.

Rarely, some people have a reaction a few hours after treatment. If you have any of the symptoms above or feel unwell after you get home, contact the hospital straight away for advice.

You are more likely to have a serious allergic reaction to cetuximab if you have an allergy to:

- red meat
- tick bites (a tick is a tiny spider-like insect found in woodland and heath areas).

You must tell your cancer doctor, nurse or pharmacist if you have either of these allergies before they give you cetuximab.

Very common side effects

These side effects happen to 10 or more people in every 100 people (10% or more) who have this treatment.

Skin changes

This treatment can cause an acne-like rash on the face, neck, chest and back. The rash is most likely to start in the first 2 or 3 weeks of treatment. Your skin may also feel dry, itchy, scaly or tender.

[Skin changes](#) are often mild but can be more severe in some people. If you notice any skin changes, contact the hospital as soon as possible on the 24-hour number you have been given. You may need creams, steroids or antibiotics. Your doctor may stop your treatment until the skin changes improve.

Here are some things you can do to take care of your skin during treatment:

- Wash with lukewarm water and mild, unperfumed, soap-free cleansers.
-

Do not use products containing alcohol on your skin, or anti-acne products (unless they have been prescribed).

- Moisturise your skin regularly with unperfumed moisturisers for dry skin conditions.
- Use sun cream of at least SPF 30 to [protect your skin from the sun](#). Cover up with clothing and a hat.

Skin changes usually improve when treatment ends.

Reduced levels of magnesium and calcium in the blood

You will have regular blood tests to check the levels of magnesium and calcium in your blood. Your doctor, nurse or pharmacist may prescribe supplements if the levels are too low.

Sore mouth and nose

The lining of your mouth and nose may become sore. You may have mouth ulcers or nosebleeds. Contact your cancer team on the 24-hour number you have been given if this happens. They can prescribe medicines to help.

Use a soft toothbrush to clean your teeth or dentures in the morning, at night and after meals.

Your nurse can give you advice on [looking after your mouth](#) and may give you a mouthwash to use.

Liver changes

Treatment with cetuximab may cause changes in the way your liver works. This is usually mild and will return to normal when the treatment finishes. You will have regular blood tests to check your liver is working properly.

Other side effects

These side effects happen to less than 10 in 100 people (less than 10%) who have this treatment. Some of them are much rarer than this but they are still important to know about. Rare means a side effect that happens to less than 1 in 1,000 people (less than 0.1%).

Diarrhoea

This treatment may cause [diarrhoea](#). Diarrhoea means passing more stools (poo) than is normal for you, or having watery or loose stools. You may also have stomach cramps. If you have a stoma, it may be more active than usual.

If you are passing loose stools 3 or more times a day and this is not normal for you, contact the hospital as soon as possible on the 24-hour number. Follow the advice they give you about:

- taking anti-diarrhoea medicines
- drinking enough fluids to keep you hydrated and to replace lost salts and minerals
- any changes to your diet that might help.

They might also ask you for a specimen of your stool to check for infection.

Feeling sick

Your doctor, nurse or pharmacist will prescribe anti-sickness drugs to help prevent or control sickness. Take the drugs exactly as they tell you to, even if you do not feel sick. It is easier to prevent sickness than to treat it after it has started.

[If you feel sick](#), take small sips of fluid often and eat small amounts regularly. It is important to drink enough fluids. If you continue to feel sick, or if you are sick (vomit) 1 to 2 times in 24 hours, contact the hospital on the 24-hour number as soon as possible. They will give you advice. They may change your anti-sickness treatment. Let them know if you still feel sick.

Serious skin changes

Rarely, this treatment can cause a serious skin reaction that needs to be treated immediately in hospital. Contact the hospital straight away on the 24-hour number if you have any of these symptoms:

- a skin rash that is spreading
- blistering or peeling skin
- flu-like symptoms, such as a high temperature and joint pain
- sores on your lips or in your mouth.

Feeling tired

[Feeling tired](#) is a common side effect of this treatment. Try to pace yourself and plan your day so you have time to rest. Gentle exercise, like short walks, can help you feel less tired.

If you feel sleepy, do not drive or use machinery.

Hair changes

This treatment may cause hair changes. You may have some gradual hair loss a few months after treatment starts. You will not usually lose all the hair on your head.

You might also notice:

- a change in the texture or colour of your hair
- more hair growth on your face or chest
- your eyelashes growing longer, or your eyebrows getting thicker.

If you are having problems with [hair changes](#), ask your cancer team for advice. Do not try to cut your eyelashes yourself. Your cancer team can arrange for an eye doctor to do this.

Any hair changes usually go back to normal after treatment ends.

Nail changes

This treatment can affect [your nails](#). They may grow more slowly or break more easily. You might notice ridges or white or dark lines across your nails. These changes usually disappear as the nails grow out after treatment. Sometimes nails can become loose or fall off. Tell your doctor, nurse or pharmacist about any changes to your nails.

If the skin around your nails becomes sore and swollen, contact the hospital straight away on the 24-hour number. These might be signs of an infection.

Sore eyes

This treatment may make your eyes dry, sore, red and itchy. Your doctor, nurse or pharmacist can prescribe eye drops to help with this.

Contact the hospital on the 24-hour number if:

- your eyes do not improve after taking the eye drops
- you have changes to your vision such as, blurred vision
- you have pain in your eye, or they feel sensitive to light
- your eye or eyelid becomes red or inflamed or feels swollen.

Headaches

This treatment may cause headaches. If you have headaches, tell your doctor, nurse or pharmacist. They can give you advice about painkillers that may help. Tell them if the headache does not get better, or gets worse.

Effects on the lungs

This treatment can cause inflammation of the lungs. This is called pneumonitis. Contact the hospital straight away on the 24-hour number if you notice any of these changes during treatment or after it ends:

- breathlessness
- a cough that does not go away
- wheezing
- a fever, with a temperature over 37.5°C.

You should also tell them if any existing breathing problems get worse. You may have:

- tests to check your lungs
- treatment, if needed.

Effects on the heart

Rarely, this treatment can affect the way the heart works. This may be more likely if you are having it with other types of anti-cancer treatment. Your doctor can tell you more about this.

You may have tests to see how well your heart is working. These may be done before, during, and sometimes after treatment.

Contact a doctor straight away if you:

- breathlessness
- dizziness
-

changes to your heartbeat (palpitations)

- swollen feet and ankles.

Other conditions can cause these symptoms, but it is important to get them checked by a doctor.

Always call **999** if you have:

- chest pain, pressure, heaviness, tightness or squeezing across the chest
- difficulty breathing.

Other important information

Blood clot risk

Cancer and some cancer treatments can increase the risk of a [blood clot](#). Contact the hospital straight away on the 24-hour number if you have any of these symptoms during or after treatment:

- throbbing pain or swelling in a leg or arm
- reddening of the skin in the area – if you have black or brown skin, this can be harder to notice, but the skin might become darker
- suddenly feeling breathless or coughing.

Always call **999** if you have:

- chest pain
- difficulty breathing.

A blood clot is serious, but it can be treated with drugs called anticoagulants. These thin the blood. Your doctor, nurse or pharmacist can give you more information about preventing and treating blood clots.

Other medicines

Some medicines can affect how this treatment works or be harmful while you are having it. Always tell your cancer doctor, nurse or pharmacist about any drugs you are taking or planning to take, such as:

- medicines you have been prescribed
- medicines you buy in a shop, pharmacy or online
- vitamins or supplements
- [herbal drugs and complementary or homeopathic therapies](#)
- recreational drugs – for example, cannabis.

Vaccinations

Cancer doctors usually recommend that people with cancer have vaccinations for flu and [coronavirus \(covid\)](#). They may also recommend other vaccines, such as Shingrix[®] for shingles. These all help reduce your risk of serious illness from these infections. Most people can have these vaccines, including people with weak immune systems.

You should not have live vaccines if your immune system is weak. This includes if you are having or recently had chemotherapy, radiotherapy or other cancer treatments that affect your immune system. Live vaccines can make you unwell because they contain a very weak version of the illness they protect you against. There are several live vaccines, including the yellow fever vaccine.

It is important to ask your doctor, nurse or pharmacist for advice about having vaccinations. They can explain what vaccines are right for you and when it is best to have them.

Contraception

Your doctor, nurse or pharmacist will advise you not to [get pregnant](#) or [make someone pregnant](#) while having this treatment and for some time afterwards. The drugs may harm a developing baby. It is important to use contraception to prevent pregnancy. Follow their advice about:

- what types of contraception to use
- how long after treatment you should continue to use contraception.

Breastfeeding

You are advised not to [breastfeed](#) while having this treatment, or for some time after treatment ends. This is because the drugs could be passed to the baby through breast milk.

Your doctor, nurse or pharmacist can give you more information.

Fertility

Some cancer drugs can affect whether you can [get pregnant](#) or [make someone pregnant](#). If you are worried about this, it is important to talk with your doctor before you start treatment.

Medical and dental treatment

If you need medical treatment for any reason other than cancer, always tell the healthcare professional that you are having cancer treatment. Give them the contact details for your cancer doctor or cancer team so they can ask for advice.

If you have appointments with a dentist, always tell them you are having cancer treatment. Talk to your cancer team before you have any dental treatment.

About our information

This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer.

References



Visit the [electronic Medicines Compendium \(eMC\)](#) to download a Patient Information Leaflet (PIL) for more detailed information. The leaflet lists all known side effects.

[Cancer Information Development team](#)

Author

Date reviewed

Reviewed: 01 October 2025 | Next review: 01 October 2027

Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

How we can help

How to print a PDF from this website

Instructions on how you can make a PDF version of this page using the print function on your browser.

Clinical Information Nurse Specialists

Our Cancer Information Nurse Specialists are dedicated cancer nurses available to talk to on our Macmillan Cancer Support Line.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

[Get help](#)